

JONATHAN FRANK, M.D.

Orthopedic Surgeon

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IN REFERENCE : JUAREZ CEJA, JUAN LUIS
DATE OF BIRTH : 07/25/1978
DATE OF INJURY : 05/06/2024
DATE OF REPORT : 12/02/2025

PERSONAL INJURY INITIAL ORTHOPEDIC CONSULTATION

CHIEF COMPLAINT

Right hip pain.

HISTORY OF PRESENT ILLNESS

A 47-year-old male who complains of right hip pain. On 05/06/2024, he was a bicyclist struck by a motor vehicle. He reports that he struck on his right side. He went to the emergency room that day where workup was negative for urgent pathology and he was sent home on pain medications. Initially, he has spinal complaints. He has been seeing a pain specialist for his spinal complaints. He reports that he started to have pain to the lateral aspect of his hip in the end of 2024. This pain is worse with standing, stairs, getting out of his car. It is better when he sleeps with a pillow between his legs. He has previously completed a course of chiropractic therapy for about 2-3 months to focus on his back. He denies previous pain or injury to his hip.

PAST MEDICAL HISTORY

None.

PAST SURGICAL HISTORY

Clavicle repair 2022.

MEDICATIONS

None.

ALLERGIES

Penicillin.

SOCIAL HISTORY

The patient works as an electronics technician. He uses tobacco products occasionally. Drinks less than 1 alcoholic beverage a day.

FAMILY HISTORY

Noncontributory.

REVIEW OF SYSTEMS

A 12-system review was positive for glasses and that noted in the HPI.

PHYSICAL EXAMINATION

GENERAL

Well-appearing gentleman, weighing 202 pounds with a height of 61 inches and is in no acute distress. Alert, cooperative, and conversant with normal affect.

MUSCULOSKELETAL

He has tenderness to palpation over the greater trochanteric bursa. Hip flexion 120 degrees, internal rotation 10 degrees, external rotation 25 degrees. No significant pain with range of motion testing. 5/5 hip abduction and adduction strength. Positive Ober. Grossly neurovascularly intact distally.

IMAGING STUDIES

An MRI of the right hip from 07/31/2025 shows a small CAM morphology. No obvious labral tear. No significant chondromalacia. Small bilateral inguinal hernias.

ASSESSMENT

A 47-year-old male with right hip trochanteric bursitis/IC tendinitis.

PLAN

I sending the patient for course of physical therapy twice a week for 6 weeks to focus on IT band/TFL stretching and abductors strengthening exercises. He was asked to follow up with me when he completes therapy. I explained to him that his hip pain complaints are likely emanating from his back injury and now having a downstream affects to his hip. All of his questions were answered.

DISCLOSURE NOTICE

The above report is for medical/legal assessment of injuries and is not to be construed as a complete physical examination for general health purposes. Only those symptoms which are believed to have been involved in the injury or that might relate to the injury have been

RE: Juarez Ceja, Juan Luis

December 2, 2025

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assessed. Regarding the general health of this patient, the patient has the right to obtain a physical examination for general purposes with a personal physician.

Notice is hereby given that pursuant to the patient's authorization and California Civil Code Section 2882.1, the undersigned claims a lien for all medical services rendered to patients on any sums that may be recovered by the patient by reason of accident of occurrence.

Electronically Approved by:

Jonathan Frank, M.D.

Director of Cartilage Restoration

California License No: A143638

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